



SURGICAL PATHOLOGY REPORT

Patient Name:

Med. Rec. #:

DOB:

(Age:

Location:

Gender:

M

Service: Urology

Ref. Physician:

Patient Address:

Accession #:

Date of Procedure:

Date of Receipt:

Date of Report:

Account #:

Billing Type:

Additional Copy to:

Ref. Source:

Clinical Diagnosis & History:

Bladder carcinoma and sigmoid mass.

Specimens Submitted:

- 1: Mesentery; biopsy
- 2: Appendix, terminal ileum, sigmoid, bladder, prostate and seminal vesicles; cystoprostatectomy and partial colectomy
- 3: Lymph nodes, left pelvic; excision
- 4: Lymph nodes, right pelvic; excision
- 5: Lymph nodes, pre-sacral; excision
- 6: Lymph nodes, right common iliac; excision
- 7: Lymph nodes, left common iliac; excision
- 8: Vas deferens, right; resection
- 9: Vas deferens left; resection
- 10: Ureter, left distal; resection
- 11: Ureter, right distal; resection
- 12: Donut, distal; excision
- 13: Donut, proximal; excision
- 14: Hernia sac; excision

ICD-O-3
Carcinoma, urothelial NOS
8120/3
Site: lymph node, pelvic
C775
JW 1/30/14

DIAGNOSIS:

1. Mesentery; biopsy:

-Benign fibrous tissue with chronic inflammation.

2. Appendix, terminal ileum, sigmoid, bladder, prostate and seminal vesicles; cystoprostatectomy and partial colectomy:

Tumor Type:

Invasive urothelial carcinoma, NOS

Histologic Grade:

High grade

Pattern of growth of the Non-Invasive component:

In situ component not identified

Pattern of growth of the Invasive component:

Inverted / Nodular

Tumor Multicentricity:

Not identified

Bladder Local Invasion:

Superficial half of muscularis propria

Extravesical Tumor Extension:

Ureters uninvolved

Urethra uninvolved

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Vascular Invasion:
Identified

Perineural Invasion:
Not identified

Surgical Margins:
Free of tumor

Non-Neoplastic Mucosa:
Exhibiting ulceration
Exhibiting foreign body reaction
Exhibiting proliferative cystitis (Brunn's nests, cystitis cystica, cystitis glandularis)

Prostate:
Other Focal HGPIN

Seminal Vesicles:
Not involved

Perivesical Lymph Nodes:
LN Not involved 13

The Pathologic Stage is (AJCC 2002):
pT2a (Invades superficial half of muscularis propria)

PR0 (No involvement of prostate)

- b. Colon with diverticulitis, diverticulosis and associated soft tissue fibrosis.
No tumor seen.
Ten benign lymph nodes (0/10).
- c. Small intestine with areas of mucosal congestion.
No tumor seen.
- d. Benign appendix.

3. Lymph nodes, left pelvic; excision:

Lymph Nodes:
Number of nodes examined:5
Number of metastatic nodes:1
The largest metastatic node is 2.1cm
Perinodal (extracapsular) extension not identified

4. Lymph nodes, right pelvic; excision:

Lymph Nodes:
Not involved
Number of nodes examined:5

5. Lymph nodes, pre-sacral; excision:

Lymph Nodes:
Not involved
Number of nodes examined:1

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6. Lymph nodes, right common iliac; excision:

Lymph Nodes:

Not involved

Number of nodes examined:5

7. Lymph nodes, left common iliac; excision:

Lymph Nodes:

Not involved

Number of nodes examined:1

8. Vas deferens, right; resection:

-Benign segment of vas deferens.

9. Vas deferens left; resection:

-Benign segment of vas deferens.

10. Ureter, left distal; resection:

-Benign segment of ureter.

11. Ureter, right distal; resection:

-Benign segment of ureter.

12. Donut, distal; excision:

-Benign colonic mucosa.

13. Donut, proximal; excision:

-Benign colonic mucosa.

14. Hernia sac; excision:

-Benign fibroadipose tissue.

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Gross Description:

1.) The specimen is received fresh for frozen section consultation, labeled "Biopsy of the mesentery" and consists of a single fragment of pink-tan soft tissue measuring 0.3 cm. Entirely submitted for frozen section.

Summary of sections:

FSC – frozen section control

2.) The specimen is received fresh, labeled "appendix, terminal ileum, sigmoid, bladder, prostate and seminal vesicles". It consists of a bladder, prostate, seminal vesicles, 13.5 x 4.0 (circumference) cm segment of terminal ileum, 24.0 x 7.0 (circumference) cm segment of colon, and 5.5 x 0.7 cm appendix. The specimen measures 18.5 cm from superior to inferior, 19.0 cm laterally and 9.0 cm from anterior to posterior. The anterior aspect of the bladder is inked black and the posterior blue. The prostatic urethral margin is shaved and submitted. The bladder is opened along the anterior midline to reveal a polypoid and focally necrotic mass associated with an area of ulceration measuring 0.5 x 2.5 x 1.5 cm which is grossly situated within the right anterior lateral wall. The lesion measures 2.7 cm from the left ureteral orifice and less than 0.1 cm from the right ureteral orifice. Both ureters are probe patent measuring up to 0.2 cm in maximum diameter. The lesion is sectioned to reveal a partially necrotic mass that extends to the anterior lateral perivesical fat. Extension to the inked fat is grossly not identified. The remaining bladder

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mucosa is focally erythematous and unremarkable. Polypoid lesions are identified in the distal prostatic urethra. Discrete perivesical lymph nodes are not grossly identified. The prostate measures 3 cm base to apex, 3.5 cm laterally, and 2.5 cm anterior to posterior. The right seminal vesicle measures 3.0 x 1.4 x 0.5 cm. The left seminal vesicle measures 2.8 x 1.5 x 0.4 cm. The prostate is serially sectioned to reveal tan cut surfaces with no masses identified. The accompanying segments of intestine are firmly adherent to the posterior aspect of the bladder wall. Cut surfaces of the fibrous areas connecting the segments of intestine show a diverticulum with peridiverticular fibrosis. The sigmoid colon is opened to show multiple polyps that range in size from 0.1 to 0.7 cm in greatest dimension. The polyps are located 3 cm from the distal margin and 21 cm from the proximal margin. The remainder of the sigmoid colon shows multiple diverticula with the remaining mucosa being unremarkable. The unoriented terminal ileum is opened to show an erythematous mucosa. Cut surfaces of the appendix are unremarkable. Multiple peri-colonic lymph nodes are identified. Representative sections are submitted including two sections from each prostatic quadrant and transition zone. TPS is submitted. Photographs are taken.

Summary of sections:

UTHM – urethral margin
RUM – right ureter margin
LUM – left ureter margin
L – lesion
RUO – right ureter orifice
LUO – left ureter orifice
LP – left posterior wall
LA – left anterior wall
RP – right posterior wall
RA – right anterior wall
TRI – trigone
DOM – dome
F – perivesical fat
RSV – right seminal vesicle
LSV – left seminal vesicle
RA – right apex prostate
LA – left apex prostate
RAM – right anterior mid prostate
RPM – right posterior mid prostate
LAM – left anterior mid prostate
LPM – left posterior mid prostate
RAB – right anterior base prostate
RPB – right posterior base prostate
LAB – left anterior base prostate
LPB – left posterior base prostate
DCM – distal colonic margin
PCM – proximal colonic margin
CP – colonic polyps
RC – random colon
TIC – diverticula
TIM – terminal ileum margins
TI – random terminal ileum
APP – appendix, representative sections
CLN – pericolic lymph nodes

3.) The specimen is received fresh, labeled "left pelvic lymph nodes" and consists of yellow-tan fatty tissue which contains five lymph nodes measuring between 0.4 and 1.4 cm in diameter. Entirely submitted.

Summary of sections:

LN1 - lymph node serially sectioned
LN2 - lymph node serially sectioned
LN3 - lymph node serially sectioned
LN4 - lymph node serially sectioned
LN5 - lymph node serially sectioned

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4.) The specimen is received fresh, labeled "left pelvic lymph nodes" and consists of yellow-tan fatty tissue which contains five lymph nodes measuring between 0.6 and 2.0 cm in diameter. Entirely submitted.

Summary of sections:

LN1 - lymph node serially sectioned
LN2 - lymph node serially bisected
LN3 - lymph node serially sectioned
LN4 - lymph node serially bisected
LN5 - lymph node serially bisected

5.) The specimen is received fresh, labeled "pre sacral lymph nodes" and consists of yellow-tan fatty tissue which contains two possible lymph nodes measuring 0.3 and 0.2 cm in diameter. Entirely submitted.

Summary of sections:

LN – two possible lymph nodes

6.) The specimen is received fresh, labeled "right common iliac lymph nodes" and consists of yellow-tan fatty tissue which contains five lymph nodes measuring between 0.3 and 1.2 cm in diameter. Entirely submitted.

Summary of sections:

LN1 - two lymph nodes
LN2 - lymph node bisected
LN3 - lymph node bisected
LN4 - lymph node bisected

7.) The specimen is received fresh, labeled "left common iliac lymph nodes" and consists of yellow-tan fatty tissue which contains one lymph node measuring 0.4 cm in diameter. Entirely submitted.

Summary of sections:

LN - one lymph node, bisected

8.) The specimen is received in formalin, labeled "right vas deferens" and consists of an unoriented 6.0 by 0.4 (diameter) cm segment of vas deferens. Entirely submitted.

Summary of sections:

E – ends
R – remainder

9.) The specimen is received in formalin, labeled "left vas deferens" and consists of an unoriented 5.1 by 0.5 (diameter) cm segment of vas deferens. Entirely submitted.

Summary of sections:

E – ends
R – remainder

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10.) The specimen is received in formalin, labeled "left distal ureter" and consists of an unoriented 1.6 by 0.5 (diameter) cm tubular fragment of soft tissue. Entirely submitted.

Summary of sections:

E -- ends

R -- remainder

11.) The specimen is received in formalin, labeled "right distal ureter" and consists of an unoriented 0.5 by 0.4 (diameter) cm tubular fragment of soft tissue. Entirely submitted.

Summary of sections:

U -- undesignated

12.) The specimen is received in formalin, labeled "distal donut" and consists of a segment of intestine closed by a 1.6 cm staple line. The other end is received open. The donut measures 1.4 cm (length) by 1.6 cm (diameter). The specimen is opened to show a 0.2 cm polyp. The polyp is located 1.0 cm from the staple line. Entirely submitted.

Summary of sections:

SL -- staple line

P -- polyp

R -- remainder

13.) The specimen is received in formalin, labeled "proximal donut" and consists of a ring of pink tan soft tissue measuring 4.5 x 1.0 x 0.5 cm. Multiple sutures and staples are attached. The mucosal surface is pink tan and focally hemorrhagic. The sutures and staples are removed and the soft tissue is entirely submitted. Also accompanying the specimen is a 4.2 x 2.9 cm nail shaped metallic device.

Summary of sections:

U--undesignated

14.) The specimen is received in formalin, labeled "hernia sac" and consists of an 8.0 x 5.0 x 1.6 cm fragment of fibroadipose tissue. No masses or lesions are identified. Representative sections are submitted.

Summary of sections:

U -- undesignated

Summary of Sections:

Part 1: Mesentery; biopsy

Block	Sect. Site	PCs
1	fsc	1

Part 2: Appendix, terminal ileum, sigmoid, bladder, prostate and seminal vesicles; cystoprostatectomy and partial colectomy

Block	Sect. Site	PCs
1	APP	4
4	CLN	4

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4	CP	9
1	DCM	1
2	DOM	4
4	F	4
6	L	6
3	LA	5
1	LAB	1
1	LAM	1
2	LP	4
1	LPB	1
1	LPM	1
1	LSV	1
1	LUM	1
1	LUO	1
1	PCM	1
3	RA	5
1	RAB	1
1	RAM	1
1	RC	1
2	RP	4
1	RPB	1
1	RPM	1
1	RSV	1
1	RUM	1
1	RUO	1
1	TI	2
3	TIC	5
1	TIM	2
2	TRI	4
1	UTHM	1

Part 3: Lymph nodes, left pelvic; excision

Block	Sect. Site	PCs
2	LN1	0
2	LN2	0
1	LN3	0
1	LN4	0
1	LN5	0

Part 4: Lymph nodes, right pelvic; excision

Block	Sect. Site	PCs
3	LN1	0
2	LN2	0
3	LN3	0
2	LN4	0
1	LN5	0

Part 5: Lymph nodes, pre-sacral; excision

Block	Sect. Site	PCs
1	LN	0

Part 6: Lymph nodes, right common iliac; excision

Block	Sect. Site	PCs
1	LN1	0
1	LN2	0
1	LN3	0

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1 LN4 0

Part 7: Lymph nodes, left common iliac; excision

Block	Sect. Site	PCs
1	LN	1

Part 8: Vas deferens, right; resection

Block	Sect. Site	PCs
1	E	2
1	R	10

Part 9: Vas deferens left; resection

Block	Sect. Site	PCs
1	E	2
1	R	10

Part 10: Ureter, left distal; resection

Block	Sect. Site	PCs
1	E	2
1	R	4

Part 11: Ureter, right distal; resection

Block	Sect. Site	PCs
1	U	2

Part 12: Donut, distal; excision

Block	Sect. Site	PCs
1	P	1
1	R	4
1	SL	1

Part 13: Donut, proximal; excision

Block	Sect. Site	PCs
2	U	10

Part 14: Hernia sac; excision

Block	Sect. Site	PCs
1	U	2

Intraoperative Consultation:

Note: The diagnoses given in this section pertain only to the tissue sample examined at the time of the intraoperative consultation.

1. FROZEN SECTION DIAGNOSIS: SP: Biopsy mesentery
PERMANENT DIAGNOSIS: SAME.

); Benign fibrous tissue with inflammation

Met for primary sent previously (same path)

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	DAF	Date Reviewed: 2/15/14